32. THE LOWER GENITAL APPARATUS

DURATION : 08:54 STUDY SHEET

COURSE SUMMARY

In this immersive video on the Lower Genital Tract, we explore the embryology of the vagina and uterus, highlighting the posterior growth dynamics that form these essential structures. Gynaecological balance is approached through the analysis of movements between the sacrum and the peritoneum, and how these interactions affect the health of the reproductive organs. Focusing on imbalances such as miscarriages or endometriosis, the video proposes fascinating therapeutic techniques based on concepts of energetic cleansing, fascial balance, and transgenerational links. This approach allows practitioners to learn how to

THE LOWER GENITAL TRACT: A BIODYNAMIC APPROACH

EMBRYOLOGY OF THE VAGINA AND UTERUS

The insertion of the **bladder** at the **Müllerian tubercle**, located in the urogenital sinus, is essential. This area includes the bladder, urethra, and the attachment to the vagina.

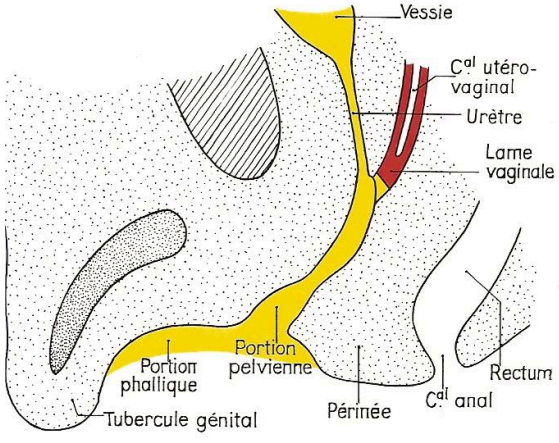


Fig. 2. — *Ouverture de la membrane uro-génitale.*

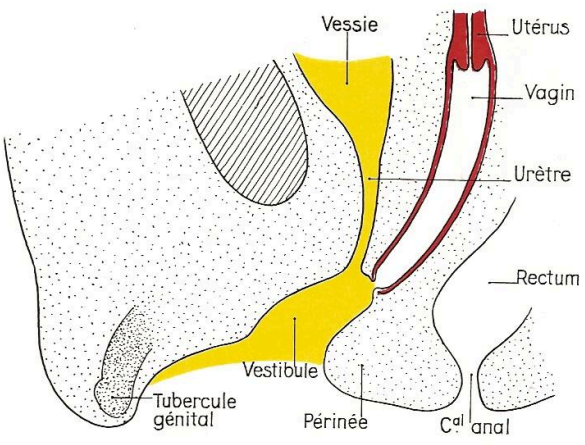


Fig. 3. — *Le vestibule définitif.*

FIG. — Development of female urogenital tract

The vagina derives from the **mesoderm**, specifically the Müllerian duct. The formation of the **vaginal plates** results from the posterior growth of the sacrum and the neural tube.

The differential growth rate of this posterior part creates the opening space of the vagina. It is the **postero-anterior** movement, between the kidney and the sacrum, that allows this extension.

The Müllerian tubercle thickens and is then stretched, not by superficial traction, but by the faster push of the posterior part. This phenomenon leads to the lengthening and formation of the vagina and cervix.

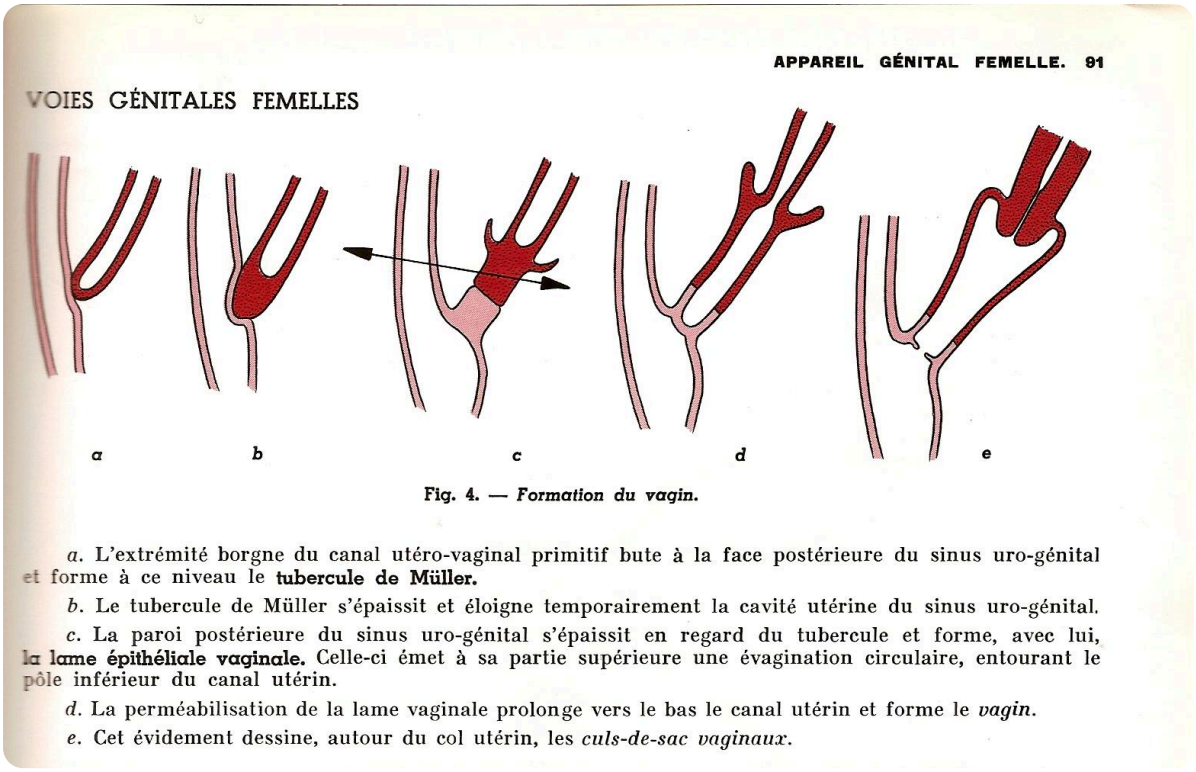


Fig. 4. — *Formation du vagin.*

a. L'extrémité borgne du canal utéro-vaginal primitif bute à la face postérieure du sinus uro-génital et forme à ce niveau le **tubercule de Müller.**

b. Le tubercule de Müller s'épaissit et éloigne temporairement la cavité utérine du sinus uro-génital.

c. La paroi postérieure du sinus uro-génital s'épaissit en regard du tubercule et forme, avec lui, **la lame épithéliale vaginale.** Celle-ci émet à sa partie supérieure une évagination circulaire, entourant le pôle inférieur du canal utérin.

d. La perméabilisation de la lame vaginale prolonge vers le bas le canal utérin et forme le *vagin*.

e. Cet évidement dessine, autour du col utérin, les *culs-de-sac vaginaux*.

FIG. — Vaginogenesis

The posterior wall of the urogenital sinus thickens, giving rise to the vaginal plates which open and stretch. This dynamic is similar, with the posterior part growing faster.

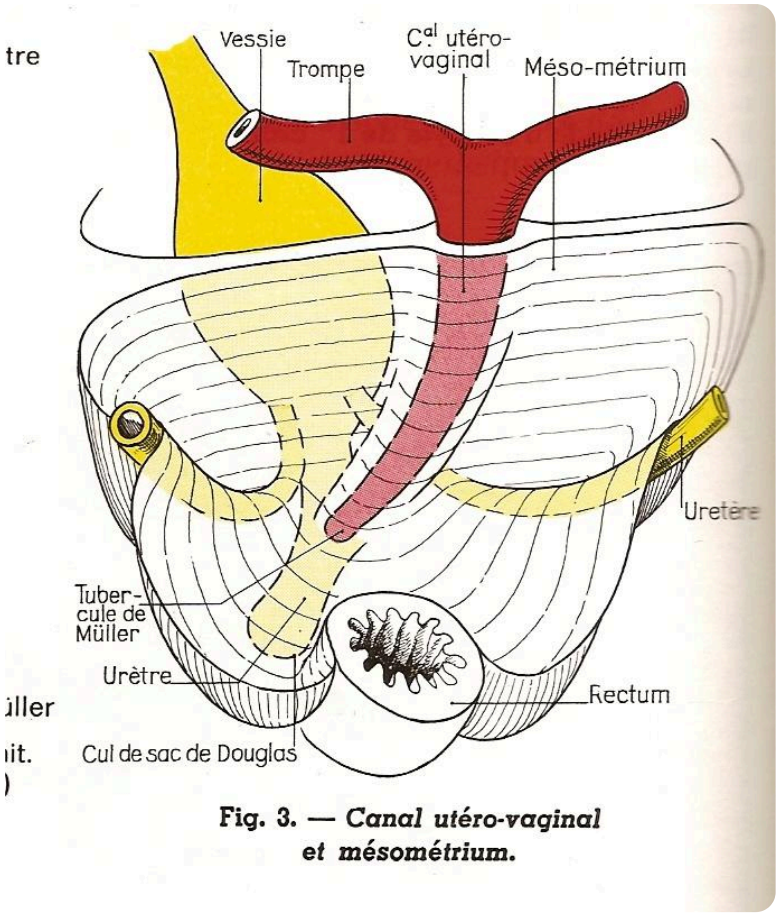


FIG. — Uterovaginal canal

GYNAECOLOGICAL BALANCE AND BIODYNAMIC MOVEMENT

During the examination of the uterus, a movement between the **sacrum** and the **peritoneum** is perceived. It is in the interaction of these two poles that vaginal and uterine balance is found.

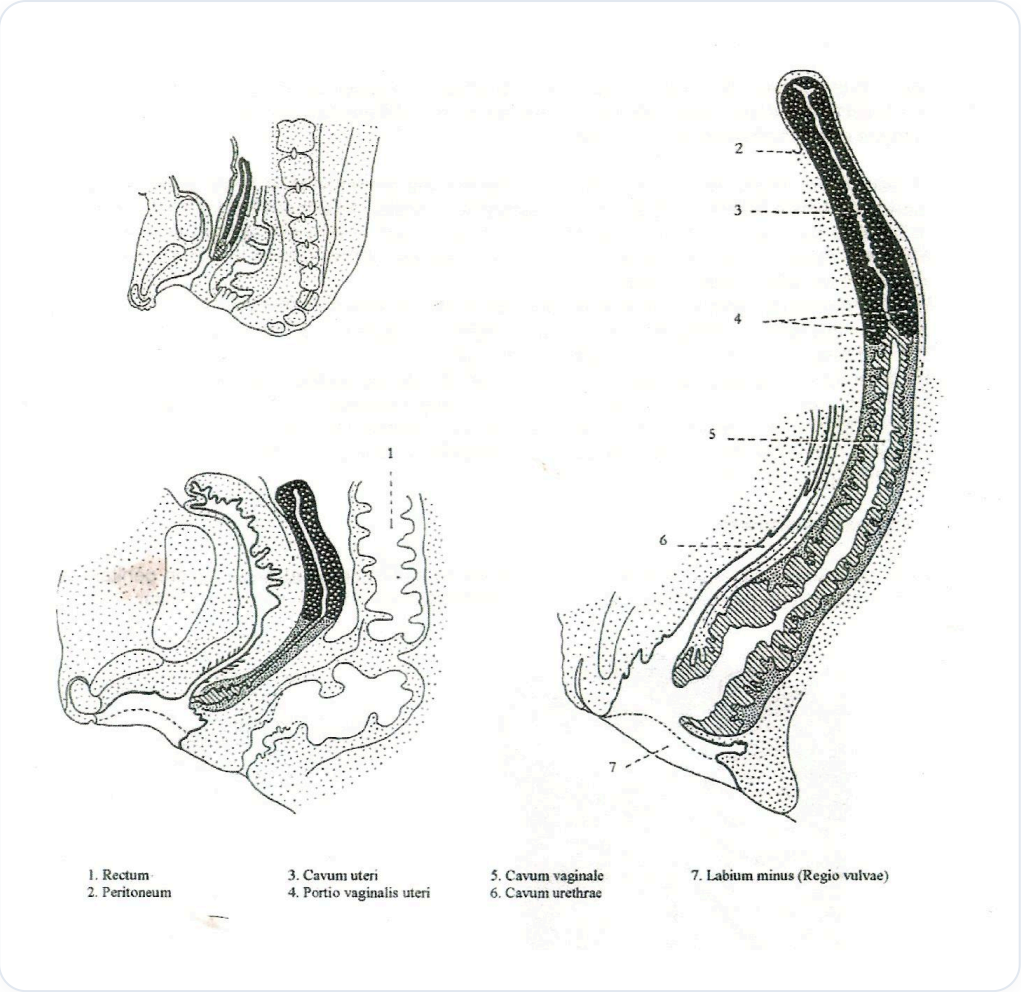


FIG. — Female uterine anatomy

At the gonadal level, an inverse movement is felt in the abdomen, following the ligament towards the pubis, like a descending wave.

The descent of the sacrum is influenced by the neural tube. Another crucial fulcrum is the combination of the pubis and the sacrum, forming an essential fascial balance of the female lesser pelvis.

This balance involves a descending line of force relative to the sacrum, influencing various functions of the female body.

THERAPEUTIC APPROACH TO UTERINE IMBALANCES

Situations such as **miscarriages** leave an energetic imprint in the uterus. Energetic cleansing of the uterus then becomes necessary.

We can approach the uterus with a **magnetic force**, learning to feel the energetic field at a distance. The practitioner perceives a frequency, a zone of heat that densifies, a sign of the energy present.

My approach consists of "scanning" and identifying the areas requiring cleansing. This involves restoring freedom to the tissues and energies.

A **retroverted uterus**, for example, can lead to congestive phases. The remote work aims to rebalance the gynaecological system, in close connection with **gastrin**.

Endometriosis, characterised by a proliferation of tissue ascending into the peritoneum, is often associated with lower back and sacroiliac pain.

THE INTRAPERITONEAL CAVITY AND THE TRANSGENERATIONAL DIMENSION

In women, there is a direct link between the cervix, the fallopian tubes, and the **intraperitoneal cavity**. This cavity is of great importance, perceived as a "sky" or a primordial space.

Each state of the body represents the best possible functional response at a given moment. This response is influenced by our lives, but also by those of our ancestors.

A retroverted uterus, for example, may be the best response the body has found to resolve a deep **transgenerational** or personal problem.

It is essential to understand why a transgenerational pattern manifests in this way. Therapy aims to decode this "best response" and offer the necessary release, if requested.

The embryo perfectly illustrates this notion: at each stage of development, it presents the best possible physiological response.

SECONDARY NEURULATION AND THE FORMATION OF THE FILUM TERMINALE

The **neural tube** is associated with a posterior caudal eminence, creating a space of light and connection. The caudal bud and the terminal part of the **notochord** are observed.

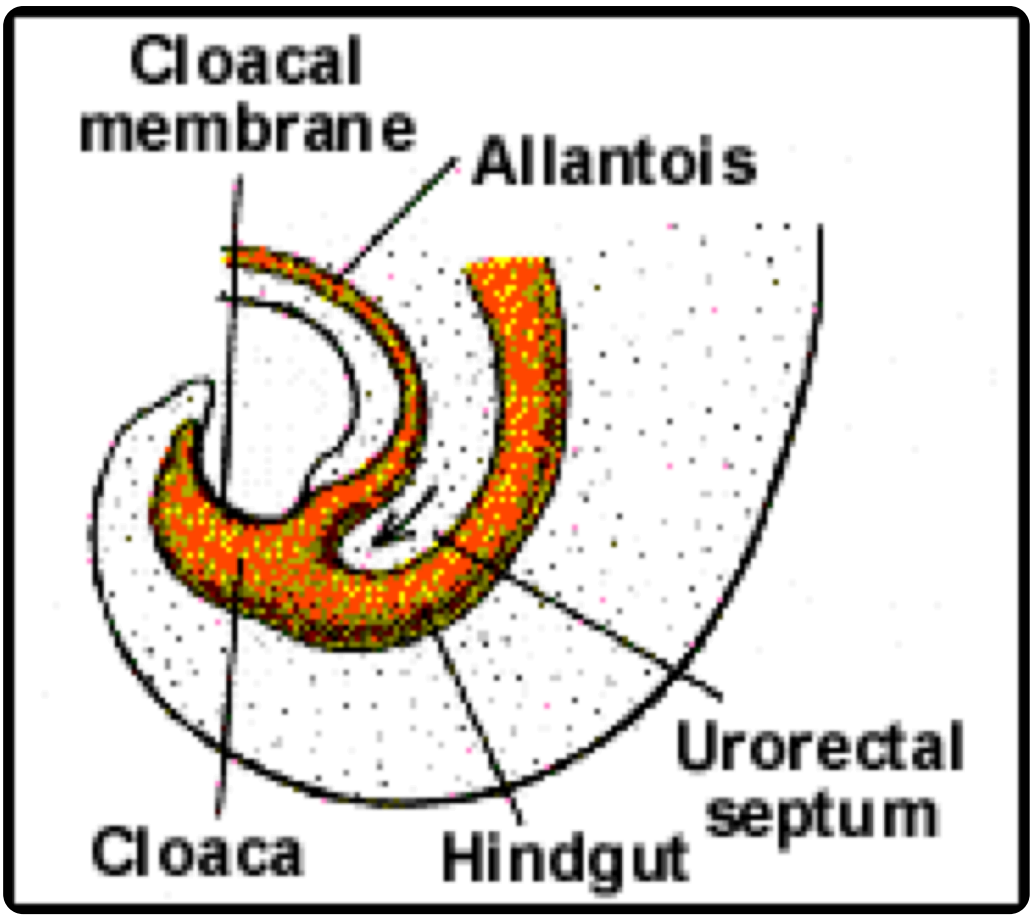


FIG. — Embryonic cloacal development

The space of the rectum and the neural tube with the posterior neuropore are in close relation. A connection forms thanks to a more pronounced developmental movement in the posterior part.

This involves a closure then a reopening to establish this connection. This process leads to the emergence of the posterior neuropore and the lengthening to form the **filum terminale**.

This is called **secondary neurulation**. Manipulation of the sacrum reveals these differential growth rates, where the initial junction lengthens posteriorly.

The integration of the caudal eminence ensures continuity with the neural tube, forming this ascending secondary neurulation.

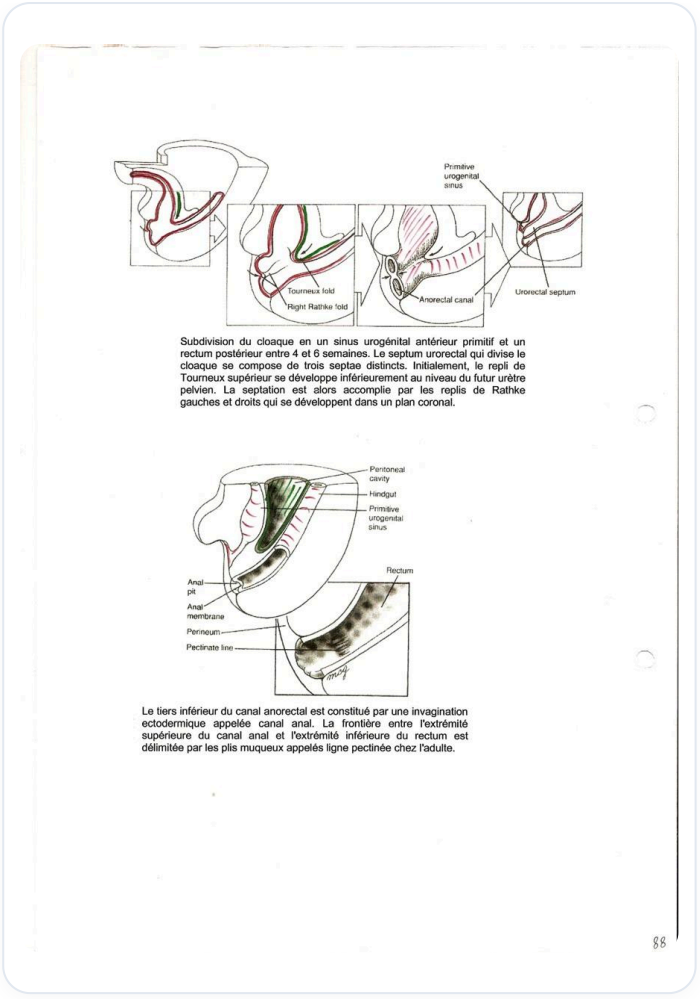


FIG. — Cloacal septation

DEVELOPMENT OF THE PERINEUM AND HEPATIC INFLUENCE

At the level of the **perineum**, we observe the emergence of the lower limbs and a traction that is exerted. This movement is deeply linked to the uterine system.

The tilting of the **liver**, in synergy with the descent of the neural tube and the sacrum, influences the developmental movement that pulls towards the linea alba, via the falciform ligament.

This hepatic development causes a tilt that affects the entire developing uterine area. It could explain phenomena of migration or differential traction related to uterine position.

A ligament develops between the pubis and the umbilicus, encapsulating the history of uterine descent and formation.

Just as the sternum contains the history of our flexion, this ventromedial ligament (on the linea alba) concentrates the history of all posterior formation, of the **gonadisation** and **uterisation** of the pelvis.

The linea alba is the meeting point of all **urogenital** history.